

Functional Assessment Report

Company Name:	Advanced OT
Therapist Name:	Stefan Barbas and Zak Azar
Therapist qualification/s:	Occupational Therapist
Type of report:	Functional Capacity Assessment Report
Date of report:	13 Feb 2026
Date of assessment	3 Feb 2026
Name of participant:	Barran Dodget
Participant's DOB:	8 Apr 1973
Participant's address:	36 Aston Martin Dr Goulburn NSW 2580, Australia
Participant's contact details:	+61 0431167907
NDIS participant number:	430938559
Assessor Declaration	I confirm that the recommendations outlined in this report have been discussed with the participant and their legal representative. This report was written in good faith and assumes that the information provided by the caregivers/participant is accurate.

BARRAN'S CURRENT NDIS GOALS

Barran's goals as stated on his NDIS plan are as follows:

- I would like to be supported to find and maintain suitable long term accommodation.
- I would like to be supported to find suitable employment in an area of interest.
- I would like to be supported to access the community and build meaningful relationships.
- I would like to be supported to maintain my mental health.

OVERVIEW OF PARTICIPANT

Background	Barran Dodget is a 52-year-old man with a diagnosis of acquired brain injury, Attention-Deficit/Hyperactivity Disorder (ADHD), schizophrenia, and adjustment disorder. An acquired brain injury can affect memory, concentration, emotional regulation, and overall cognitive functioning,
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often resulting in reduced independence and difficulty coping with everyday demands. ADHD is associated with difficulties in attention, impulse control, organisation, and emotional regulation, which can further impair daily functioning. Schizophrenia is a severe mental illness characterised by disturbances in thinking, perception, mood, and behaviour, and include symptoms such as paranoia, hallucinations, disorganised thinking, and social withdrawal. Adjustment disorder occurs in response to significant life stressors and can result in marked emotional distress, anxiety, depression, and reduced ability to cope. The combined impact of these conditions significantly affects Barran's ability to manage daily life, maintain stable accommodation, engage in employment, and sustain social and emotional wellbeing.

Barran was born on 8 April 1973 and is an adult male currently residing in Goulburn. He has experienced a highly complex and unstable life course marked by longstanding mental health challenges, psychological trauma, and systemic barriers to accessing consistent and appropriate support. Barran reports limited family involvement and minimal ongoing family support, which has contributed to chronic social isolation and a lack of reliable personal support networks. Over time, he has lived in multiple environments, including periods of instability and recent shared or supported accommodation arrangements that he describes as unsafe, distressing, and destabilising, further exacerbating his mental health symptoms.

A significant recent life event includes a marked deterioration in mental health that resulted in an admission to Goulburn Hospital. This admission followed escalating psychological distress, feelings of entrapment, and experiences of perceived targeting and abuse within his living environment. Barran reports a history of a suicide attempt associated with severe psychological trauma, which resulted in a subsequent brain injury. He describes this injury as having a profound and enduring impact on his cognition, sense of identity, emotional regulation, and overall functional capacity.

	Despite his current difficulties, Barran is highly articulate and intellectually capable. He reports completing tertiary-level education and holds a PhD, with academic interests in philosophy, ethics, and artificial intelligence. He has previously functioned at a high level and has worked as a mental health advocate and professional writer. Notably, he authored an autobiography detailing his lived experience of schizophrenia and recovery, reflecting periods of meaningful engagement, insight, and contribution to the community. At present, however, his ability to engage in employment or volunteering is significantly compromised due to severe mental health instability, cognitive impairment related to brain injury, financial hardship, and a lack of consistent support. Barran identifies a strong emotional attachment to his companion dog, Crystal, whom he describes as a key protective factor. Crystal provides emotional comfort, grounding, companionship, and a sense of responsibility, which supports Barran's emotional regulation and reduces feelings of isolation. Barran also reports a significant and meaningful relationship with his fiancé, who resides in Sydney. He identifies this relationship as a major protective factor for his mental health and overall wellbeing. However, he reports that contact and potential relocation have been obstructed by systemic and guardianship-related barriers. These barriers have contributed to intense emotional distress, loneliness, feelings of powerlessness, and further deterioration in his mental health and daily functioning.
Disability Overview	Barran presents with a complex psychosocial and neurological disability profile that significantly affects his daily functioning and overall quality of life. He reports an acquired brain injury following a suicide attempt, which he describes as a near-death experience. This brain injury has resulted in ongoing and persistent difficulties with memory, concentration, executive functioning, emotional regulation, and tolerance to stress. These cognitive and emotional impairments limit his ability to process information, manage daily tasks, cope with change, and respond effectively to challenging situations, particularly during periods of heightened stress.

	Barran has a longstanding diagnosis of schizophrenia, which continues to impact his perception, thought processes, insight, and emotional stability. He experiences symptoms that interfere with his ability to accurately interpret situations, trust others, and maintain a consistent sense of safety. In addition, he reports features consistent with adjustment disorder, particularly in response to prolonged exposure to stress, loss of personal autonomy, and ongoing systemic adversity. These stressors have contributed to emotional distress, reduced coping capacity, and worsening mental health symptoms over time. Barran also has a diagnosis of Attention Deficit Hyperactivity Disorder (ADHD), which contributes to difficulties with attention, task initiation, organisation, and sustaining goal-directed activity. These challenges further impair his ability to manage routines, complete tasks independently, and follow through with plans or appointments. The combined impact of ADHD and brain injury significantly reduces his executive functioning and ability to maintain consistency in daily life. Barran reports periods of significant paranoia, distress, and fear, particularly when living in environments where he feels unsafe, controlled, or unheard. These experiences intensify his mental health symptoms and lead to increased emotional dysregulation, withdrawal, and difficulty trusting support systems. His mental health conditions interact with the effects of his brain injury, resulting in marked functional impairment across multiple areas, including daily living skills, communication, decision-making, self-advocacy, and emotional regulation. Recent deterioration in Barran's functioning has been associated with social isolation, limited access to therapeutic support, financial restriction, and perceived coercive control through external systems. These factors have compounded his existing disabilities, leading to increased psychological distress and reduced capacity to stabilise or recover without appropriate, consistent, and person-centred support.
Medical Conditions	• Acquired brain injury following a suicide attempt • Schizophrenia with ongoing psychosocial disability

	• Adjustment disorder related to prolonged stress, trauma, and loss of autonomy • Attention Deficit Hyperactivity Disorder (ADHD) • Reported history of hospital admissions related to mental health crises • Ongoing sleep disturbance, high stress levels, and emotional dysregulation secondary to mental health conditions
Informal Supports	• Barran reports no reliable informal support in his current location, contributing to social isolation and reduced day-to-day assistance. • Barran identifies his fiancé in Sydney as his primary source of emotional support; however, disrupted access to this relationship has significantly reduced protective factors and emotional stability. • Barran's companion dog, Crystal, provides ongoing emotional comfort, grounding, and a sense of responsibility, but does not replace the need for human support or practical assistance.
Formal Supports	• Barran currently receives approximately 2 hours per day of NDIS-funded support worker assistance. • This level of support is significantly below recommendations outlined in previous plans and professional reports. • Barran is not currently engaged with a psychologist, psychiatrist, counsellor, or other allied health professionals. • Barran reports no access to consistent mental health therapy, despite presenting with high clinical risk and complex support needs. • Current formal support is insufficient to maintain safety, stability, or meaningful functional participation, and the limited assistance provided does not adequately meet Barran's needs.

CURRENT HOME AND LIVING ENVIRONMENT	
Current Living Arrangements	• Barran currently resides in Supported Independent Living (SIL) accommodation, despite not being formally approved for SIL support.

	- Barran pays rent for this accommodation arrangement without receiving the full structure or protections typically associated with approved SIL funding. - This living arrangement creates uncertainty and instability, as the level of support provided does not align with Barran's assessed needs or the expectations of a SIL environment.
Accessibility	No issues were reported or observed.

STANDARDISED ASSESSMENT RESULTS:

WHODAS:

WHODAS 2.0 RESULTS		
Domain	Score	Level of Difficulty / Disability
Understanding and Communicating	33.33%	Mild
Getting Around	40.00%	Mild
Self-Care	81.25%	Severe
Getting Along with People	75.00%	Severe
Life Activities	84.38%	Severe
Participation in Society	78.13%	Severe
Total disability 66.70%		

The World Health Organisation Disability Assessment Schedule 2.0 (WHODAS 2.0) is a standardised assessment tool used to measure how health conditions affect a person's ability to function in daily life. It looks at functioning across six key life domains and reflects the level of difficulty experienced over the past 30 days.

- Understanding and Communicating – Mild Difficulty (33.33%):** Barran reports mild difficulty with understanding information, concentrating, learning new tasks, and communicating with others. While these challenges are present, Barran is generally able to understand and express themselves, particularly with familiar tasks or adequate time and support.
- Getting Around – Mild Difficulty (40.00%):** This score suggests mild limitations with physical mobility, such as standing for long periods, moving

around outside the home, or travelling distances. These difficulties cause some inconvenience or fatigue but do not appear to be severely restrictive at this time.

3. **Self-Care – Severe Difficulty (81.25%):** Barran reports severe difficulty with self-care activities, including personal hygiene, dressing, eating, or managing basic daily routines. This indicates a significant impact on Barran's independence and suggests that support be required to maintain personal care and daily wellbeing.
4. **Getting Along with People – Severe Difficulty (75.00%):** This result indicates severe challenges in social interactions and relationships. Barran experiences difficulty engaging with others, managing conflict, maintaining relationships, or feeling comfortable in social situations. These difficulties contribute to social withdrawal or isolation.
5. **Life Activities – Severe Difficulty (84.38%):** Barran reports severe difficulty managing everyday responsibilities, such as household tasks, work, study, or other routine activities. This suggests that Barran's functional capacity in structured or demanding environments is significantly limited and requires ongoing support or adjustments.
6. **Participation in Society – Severe Difficulty (78.13%):** This score reflects severe restrictions in participating in community activities, social roles, and broader societal involvement. Barran feels unable to engage fully in community life due to emotional, psychological, or functional barriers.

GAD-7_Anxiety

The Generalised Anxiety Disorder 7-item scale (GAD-7) is a self-report screening tool used to assess the severity of anxiety symptoms over the past two weeks. Higher scores indicate greater levels of anxiety and functional impact.

Barran's score of 19 falls within the severe range of anxiety. This indicates that Barran is experiencing a high level of ongoing anxiety that is persistent and difficult to control. Symptoms commonly associated with this level of anxiety include excessive and uncontrollable worry, restlessness, feeling on edge, difficulty relaxing, irritability, muscle tension, and sleep disturbance. At this severity level, anxiety is affecting Barran's ability to concentrate, make decisions, and manage everyday tasks. Barran feels overwhelmed by daily responsibilities and

experiences a constant sense of fear or anticipation, even in situations that are not objectively threatening.

LSP-16:

All items are answered on an anchored four-point scale, with higher scores indicating a greater degree of disability. The total score on the LSP-16 can range from 0 to 48.

Subscale and brief item name	Subscale scores
A. Withdrawal • Difficulty in conversation • Withdraw from social contact • Shows warmth • Maintain friendships	9/12
B. Self-care • Well-groomed • Clean clothes • Neglect health • Adequate diet • Work capability	10/15
C. Compliance • Look after own prescribed medication • Willing to take prescribe medication • Cooperate with health services	6/9
D. Anti-social • Violent • Problems with others • Offensive behaviour • Irresponsible behaviour	6/12
Total score	31/48

The Life Skills Profile – 16 (LSP-16) is a brief measure used to assess a person's level of functioning in everyday life. It focuses on practical life skills, social functioning, self-care, and behaviours that affect independent living. Higher scores indicate greater levels of difficulty.

1. **Withdrawal – High Difficulty (9 / 12):** This score suggests significant withdrawal from social interaction and daily engagement. Barran tend to isolate themselves, show reduced motivation to engage with others, or have limited involvement in everyday activities. This level of withdrawal affects relationships and reduces opportunities for social support.
2. **Self-Care – High Difficulty (10 / 15):** Barran shows notable difficulty with self-care tasks, such as personal hygiene, daily routines, and maintaining basic living skills. This indicates reduced independence and suggests that Barran requires support to manage day-to-day self-care and personal responsibilities.
3. **Compliance – Moderate Difficulty (6 / 9):** This score reflects moderate difficulty following instructions, routines, or agreed expectations. Barran struggles at times with structure, rules, or consistency, particularly when feeling stressed or overwhelmed. These challenges affect engagement with services or daily responsibilities.
4. **Anti-Social Behaviour – Moderate Difficulty (6 / 12):** Barran demonstrates moderate difficulties related to behaviour that disrupt social or community functioning. This includes irritability, impulsivity, or difficulty managing emotions in social situations. While these behaviours are present, they are not at the most severe level.

DASS:

The Depression Anxiety Stress Scales – 21 item version (DASS-21) is a self-report measure designed to assess the severity of symptoms related to depression, anxiety, and stress over the past week. Each subscale consists of 7 items, with higher scores indicating greater symptom severity.

1. **Depression-Extremely Severe (19 / 21):** Barran's depression score falls within the Extremely Severe range. This suggests the presence of significant depressive symptoms, which include persistent low mood, feelings of hopelessness, reduced motivation, loss of interest or pleasure in activities,

low self-worth, and possible difficulties with concentration and decision-making. The severity of this score indicates that depressive symptoms are having a substantial impact on Barran's emotional wellbeing and daily functioning.

2. **Anxiety-Extremely Severe (19 / 21):** The anxiety score is also within the Extremely Severe range, indicating pronounced anxiety symptoms. This reflects intense physiological arousal, excessive worry, fear, panic-like symptoms, restlessness, or heightened alertness. Such a score suggests that anxiety is pervasive and significantly interferes with Barran's ability to feel calm, safe, and in control across daily situations.
3. **Stress-Extremely Severe (19 / 21):** Barran's stress score falls in the Extremely Severe range, indicating high levels of chronic tension and difficulty coping with demands. This can be associated with persistent irritability, agitation, feeling overwhelmed, difficulty relaxing, and reduced stress tolerance. The score suggests that Barran is experiencing considerable strain that is affecting emotional regulation and overall resilience.

FUNCTIONAL CAPACITY ASSESSMENT – INTERVIEW/OBSERVATIONAL ASSESSMENT:

PHYSICAL	
Mobility/Functional Movement	• Barran is physically able to mobilise independently within his home and community and does not require mobility aids. • Barran reports a significant motor vehicle accident in 2020, which has resulted in ongoing physical discomfort and reduced functional capacity. • Since the accident, Barran experiences reduced endurance, strength, and confidence with movement, particularly in unfamiliar or unpredictable environments. • Although ambulant, Barran's mobility is functionally limited by the interaction between physical discomfort and his psychosocial and neurological disabilities, including acquired brain injury, schizophrenia, and anxiety-related symptoms.

	• Barran reports hypervigilance, fear, and psychological distress when outside the home, especially in environments where he feels unsafe or at risk of confrontation. • He avoids certain outdoor and community environments due to perceived threats and emotional safety concerns, rather than purely physical barriers. • Barran fatigues more easily than previously and has a reduced tolerance for sustained physical activity, impacting his ability to engage in work-related or community-based activities. • Mental health instability, combined with the effects of brain injury, significantly limits his ability to move confidently, plan outings, and navigate environments independently. • Barran's capacity to mobilise safely and effectively is highly dependent on predictable, emotionally safe, and supported environments, highlighting the functional impact of his disability on mobility rather than a primary physical impairment.
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PSYCHOLOGICAL	
Anxiety	Barran experiences pervasive and severe anxiety that significantly affects his day-to-day functioning and overall sense of safety. His anxiety is strongly triggered by interactions with government and administrative systems, particularly the NDIS, the Public Guardian, and financial control mechanisms, which he experiences as overwhelming, disempowering, and threatening. These interactions often increase feelings of loss of control and fear, leading to heightened distress and reduced capacity to engage effectively with required processes. Barran reports heightened anxiety in unfamiliar environments, around unfamiliar people, and when required to navigate bureaucratic or administrative tasks independently. He describes a constant state of hypervigilance and fear in his current living environment, stating that he does not feel safe leaving the house or engaging freely in the community.

	This ongoing sense of threat contributes to a persistent feeling of being on edge and unable to relax. Anxiety significantly limits Barran's ability to engage in work, study, appointments, and community participation, often resulting in avoidance, withdrawal, and social isolation. He reports experiencing ongoing physiological symptoms, including persistent restlessness, racing thoughts, heightened arousal, sleep disturbance, and an inability to feel calm. When anxiety intensifies, Barran tends to withdraw and ruminate, becoming increasingly distressed rather than being able to self-soothe or regulate his emotions.
Trauma/ Mental Health	Barran presents with a complex trauma and mental health profile shaped by longstanding psychological distress and significant adverse life experiences. He reports a history of a suicide attempt associated with profound psychological trauma, which resulted in an acquired brain injury. This event has had lasting effects on his mental health, emotional regulation, and sense of personal safety. Barran describes repeated experiences of feeling unsafe, targeted, controlled, and unheard, which have compounded his trauma over time and contributed to ongoing deterioration in his mental health. Barran has a longstanding diagnosis of schizophrenia, which significantly impacts his perception, thought processes, emotional regulation, and tolerance to stress. These symptoms reduce his ability to accurately interpret situations, manage emotional responses, and feel secure in his environment. In addition, he presents with adjustment-related mental health difficulties in response to prolonged stress, loss of autonomy, financial restriction, and ongoing systemic adversity. These factors have further reduced his coping capacity and intensified feelings of helplessness and distress. Barran reports repeated hospital admissions related to mental health crises, including a recent admission to Goulburn Hospital following a significant decline in mental health. Despite the severity and chronic nature of his difficulties, he is not currently engaged with a psychiatrist,

	psychologist, counsellor, or other mental health clinician, leaving his complex needs largely unsupported. Trauma-related stress responses reported by Barran include heightened fear, mistrust, hypervigilance, sleep disturbance, and difficulty feeling safe, even within his own home.
Moods	Barran reports significant fluctuations in mood, characterised by periods of intense emotional distress, fear, and feelings of hopelessness. These mood changes are not brief or situational, but are closely connected to ongoing stressors in his life and have a substantial impact on his emotional wellbeing. At times, he describes experiencing emotional numbness and exhaustion, during which his motivation, energy, and emotional responsiveness are markedly reduced. During these periods, he finds it difficult to engage with daily activities or derive a sense of purpose or pleasure. Barran's mood instability is strongly linked to environmental and situational stressors, including financial hardship, social isolation, limited access to appropriate supports, and prolonged separation from his fiancé. These stressors significantly increase emotional strain and contribute to a rapid deterioration in mood. When stressors escalate, Barran reports that his mood declines quickly, resulting in reduced functional capacity, particularly in areas such as self-care, organisation, and engagement with others or services. Barran also reports persistent feelings of being trapped and powerless, which further intensify depressive symptoms and emotional exhaustion. These feelings contribute to a sense of overwhelm and hopelessness, making it increasingly difficult for him to cope with demands or envision improvement.
Volition	Barran has historically demonstrated strong volition, with a clear sense of purpose and intrinsic motivation, particularly through activities such as writing, mental health advocacy, philosophy, and art. In the past, he worked as a mental health advocate and authored an autobiography describing his lived experience of mental illness and recovery. These achievements reflect high levels of insight, creativity, and engagement during periods

	when he was adequately supported and had access to stable environments and resources. At present, Barran's volition is significantly impaired by ongoing mental health instability, cognitive fatigue associated with brain injury, limited access to resources, and a living environment that he experiences as unsafe and distressing. He reports marked difficulty initiating and sustaining activities, even those that are personally meaningful, due to persistent exhaustion, anxiety, and emotional distress. Tasks that previously provided a sense of identity and motivation now feel overwhelming or unattainable. The inability to access basic tools, such as a computer, phone, or a stable and predictable environment, has further eroded Barran's sense of purpose, autonomy, and agency. While his relationship with his fiancé and his care for his companion dog remain key sources of meaning and motivation, prolonged separation from these supports has significantly reduced his hope and drive. As a result, Barran's current level of volition is markedly diminished, limiting his capacity to engage in goal-directed activity without substantial external support and stability.
Emotional Regulation	Barran demonstrates significantly impaired emotional regulation associated with the combined effects of schizophrenia, acquired brain injury, ADHD, and prolonged exposure to trauma. His emotional responses tend to escalate rapidly when he perceives threat, injustice, loss of control, or invalidation, particularly during interactions with government systems, financial management bodies, and service providers. In these situations, emotional arousal increases quickly and becomes difficult for him to manage or contain. When emotionally overwhelmed, Barran reports an inability to calm himself independently. Rather than resolving over time, distress often intensifies into fear, panic, and cognitive overload. This emotional dysregulation is further intensified by chronic hypervigilance, disrupted sleep, and ongoing environmental stress, including living in an environment he does not perceive as safe. These factors reduce his capacity to recover from

	emotional distress and increase the frequency and severity of dysregulated episodes. Barran reports becoming emotionally flooded when required to engage with NDIS processes, Public Guardian restrictions, or financial hardship. These stressors often lead to shutdown, withdrawal, or crisis-level distress. During periods of emotional dysregulation, he struggles to complete basic activities of daily living, such as eating, sleeping, maintaining routines, or attending appointments, resulting in further functional decline. Emotional regulation difficulties are compounded by a lack of therapeutic input, as Barran currently has no access to psychology, psychiatry, or counselling services to support the development of coping strategies, grounding skills, or emotional containment. As a result, he relies heavily on external protective factors, including proximity to trusted individuals and his companion dog, to remain emotionally regulated. Disruption or removal of these supports leads to rapid destabilisation. Barran's emotional regulation capacity improves when he feels believed, safe, supported, and has predictable routines, highlighting the strong environmental dependence of his emotional functioning.
Thought Processes	Barran presents disorganised and distress-driven thought processes during periods of heightened stress, consistent with the combined impact of schizophrenia and trauma-related cognitive patterns. He reports persistent fears regarding his personal safety, including beliefs that he is being targeted, surveilled, or threatened across multiple locations. Barran describes receiving death threats and expresses strong fear that his phone and communications have been compromised. These beliefs contribute to pervasive mistrust, heightened anxiety, and a constant sense of danger. These thought patterns result in chronic hypervigilance, difficulty concentrating, and an inability to feel safe, even within his own home. Barran reports significant distress when his concerns are perceived as being dismissed or attributed solely to paranoia by professionals, rather than being explored through a trauma-informed or risk-informed

	approach. This experience of feeling disbelieved further reinforces mistrust and emotional distress, worsening his mental state. Barran's acquired brain injury further impairs his ability to filter information, tolerate stress, and reality-test effectively when under pressure. As a result, fearful thoughts become more intense, persistent, and difficult to challenge. When overwhelmed, Barran experiences rumination and fixation on perceived threats, financial injustice, and systemic barriers, which escalates distress and significantly reduces his functional capacity. These disrupted thought processes interfere with decision-making, problem-solving, communication with services, and self-advocacy, limiting his ability to engage effectively with supports or pursue his needs. Barran reports that his thinking becomes more coherent and organised when he feels emotionally contained, supported by trusted individuals, and removed from environments he perceives as threatening. This highlights the strong influence of emotional safety and support on his cognitive functioning.
Behavioural Concerns	Barran's behavioural concerns are primarily internalised responses to distress, rather than outward aggression toward others. He does not present with violent or threatening behaviour. Instead, when experiencing emotional or cognitive overwhelm, his behaviour is characterised by withdrawal, isolation, avoidance, and shutdown. During these periods, Barran disengages from daily routines, neglects self-care, and remains socially isolated for extended periods of time, which further exacerbates his mental health difficulties. Barran has a documented history of self-harm and a suicide attempt during times of extreme psychological distress, indicating a significant risk profile when emotional regulation is compromised and supports are insufficient. Current behavioural risk is elevated due to prolonged isolation, lack of therapeutic intervention, financial restriction, and an ongoing perception that his living environment is unsafe. These factors interact to increase vulnerability to deterioration and crisis presentation.

	Barran reports difficulty trusting service providers and systems, stemming from repeated experiences of feeling invalidated or dismissed. As a result, he avoids engagement with services or delays help-seeking until distress becomes severe. When overwhelmed, Barran communicates in a highly distressed manner, including sending lengthy written statements or urgently seeking reassurance. These behaviours reflect attempts to regain a sense of control, safety, and understanding rather than deliberate disruption. Behavioural functioning improves when Barran has consistent support worker presence, access to advocacy, and a sense of agency over decisions that affect his life. His behavioural risks are closely linked to environmental instability and unmet support needs, rather than non-compliance or oppositional behaviour. Without increased structured support, appropriate therapeutic input, and relocation to a psychologically safe and stable environment, Barran remains at risk of further behavioural withdrawal, functional decline, and recurrent mental health crises.
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COGNITION	
Memory	Barran presents with significant memory impairment consistent with the effects of an acquired brain injury and chronic mental illness. He reports ongoing difficulty retaining new information, including remembering appointments, deadlines, conversations, and instructions. These memory difficulties interfere with his ability to manage daily responsibilities independently and increase his reliance on external support. Memory impairment is significantly worsened during periods of emotional distress, heightened anxiety, and sleep deprivation, further reducing his capacity to process and recall information when under stress. Barran also reports losing access to emails, documents, and administrative records due to financial hardship and loss of technology. This has compounded his memory difficulties by removing external memory aids that previously supported organisation and recall.

	As a result, Barran struggles to track important tasks such as bill payments, correspondence with services, and required follow-up actions. These difficulties have led to missed appointments, delayed responses, and lost opportunities for support, which in turn increase stress and emotional overwhelm. Barran reports that while memory for emotionally charged or distressing events remains vivid and persistent, practical day-to-day information is easily forgotten, reflecting a trauma-influenced memory pattern that further impacts his functional capacity and ability to manage everyday life.
Attention/ Concentration	Barran demonstrates significantly reduced attention span and difficulty sustaining concentration, consistent with the combined effects of ADHD and an acquired brain injury. He becomes easily overwhelmed when faced with complex tasks, extensive paperwork, or processes that involve multiple steps, which limits his ability to complete tasks independently and efficiently. Barran reports that his concentration deteriorates markedly in high-stress environments, particularly when dealing with NDIS processes, Public Guardian matters, or situations involving perceived threats to his safety. In these contexts, cognitive overload occurs quickly, making it difficult for him to organise his thoughts or remain focused. He reports difficulty maintaining attention long enough to complete forms, read lengthy documents, or participate effectively in phone or online appointments, often requiring tasks to be paused or abandoned. Fatigue, anxiety, and chronic hypervigilance further reduce Barran's capacity to maintain attention across daily activities. As a result, sustained concentration is inconsistent and heavily dependent on emotional safety, low-stimulation environments, and external support to manage tasks that require focus, organisation, and cognitive endurance.
Problem Solving/ Decision Making	Barran experiences significant difficulty with problem solving and decision making, particularly when under pressure or faced with emotionally charged situations. When required to make complex decisions, he becomes easily overwhelmed and freeze, ruminate, or disengage rather than move

	toward resolution. This response limits his ability to respond effectively to challenges and contributes to delays or avoidance in important areas of his life. Impairment in executive functioning reduces Barran's capacity to prioritise tasks, break problems into manageable steps, or evaluate options in a clear and objective way. Decision making is often driven by fear, perceived threat, and emotional distress, rather than logical sequencing or careful consideration of outcomes. As a result, choices are delayed, avoided, or experienced as distressing rather than empowering. Barran requires external support to navigate decisions related to accommodation, finances, health care, and engagement with services. Without structured assistance and guidance, decision-making demands can quickly become overwhelming, leading to increased stress, reduced functioning, and further withdrawal.
Task Initiation and Completion	Barran's task initiation and completion are significantly affected by fluctuating insight, mental state, and perceptions of environmental safety. His insight varies depending on his level of distress and whether he feels emotionally and physically safe. During periods of heightened stress, his ability to assess risk, interpret others' intent, and judge situations accurately is substantially compromised. This makes it difficult for him to confidently start tasks or follow them through to completion. Barran reports feeling targeted, surveilled, or unsafe, which strongly influences his judgement and behaviour. When these perceptions are present, attention and energy are diverted toward managing fear and monitoring perceived threats, rather than engaging in practical or goal-directed tasks. His diagnoses of schizophrenia and a history of trauma further impair his capacity to reality test effectively when under stress, increasing uncertainty and self-doubt. As a result, Barran is vulnerable to fear-driven decision making, which often leads to avoidance, hesitation, or abandonment of tasks once initiated. Reduced confidence in navigating situations independently limits his ability

	to organise activities, maintain routines, and complete everyday responsibilities. Task initiation and completion improve when Barran feels safe, supported, and emotionally contained, highlighting the strong impact of mental health and environmental factors on his functional capacity.
Insight/ Judgment	Barran demonstrates historically strong insight and judgement, supported by high premorbid intellectual capacity and a significant record of academic achievement. However, his current insight and judgement are inconsistently applied due to the combined effects of cognitive fatigue, memory impairment, emotional dysregulation, and a lack of stable and reliable support. While he retains the ability to reflect on his situation during periods of stability, this capacity is reduced when he is fatigued, distressed, or overwhelmed. Barran experiences difficulty learning and adapting to new systems, routines, or technologies without hands-on guidance and repeated support. His reduced capacity to absorb and retain new information limits his ability to make informed judgements when navigating unfamiliar or complex situations independently. Learning is most effective when information is clearly structured, broken down into manageable steps, repeated over time, provided in written form, and reinforced by trusted support people who can assist with interpretation and application. Without ongoing structured support, Barran is unable to sustain new learning or adapt to change, which negatively affects his judgement in practical decision-making. As a result, his insight and judgement are highly dependent on environmental stability, clear communication, and consistent assistance, rather than a lack of intelligence or willingness to engage.
Learning	Barran's current learning capacity is significantly reduced, despite a history of strong premorbid intellectual ability. Cognitive fatigue, memory impairment, attentional difficulties, and emotional dysregulation limit his ability to absorb, retain, and apply new information. These factors reduce his capacity to learn independently and make it difficult for him to keep up with changes or new demands.

	Barran struggles to learn new systems, routines, technologies, and administrative processes without hands-on guidance, repetition, and ongoing reinforcement. Learning breaks down quickly when information is complex, delivered verbally only, or requires independent follow-through. In these situations, he becomes overwhelmed and is unable to consolidate new knowledge effectively. Barran benefits most when information is simplified, broken into small and manageable steps, provided in writing, and revisited multiple times. Learning is most effective when reinforced by trusted support people who can provide reassurance, clarify misunderstandings, and assist with practical application in real-world settings. Without structured and consistent support, learning new skills or adapting to change is not sustainable, often leading to frustration, withdrawal, and further functional decline.
Safety Awareness	Barran's safety awareness is significantly impaired due to the combined effects of cognitive impairment, perceptual disturbance, trauma-related hypervigilance, and emotional dysregulation. These factors interact to reduce his ability to accurately assess risk and respond appropriately to unsafe situations, particularly when he is under stress or emotionally overwhelmed. Barran struggles to distinguish between real and perceived danger, as fear, paranoia, and heightened threat perception interfere with judgement. This results in difficulty evaluating personal safety risks in community environments, unfamiliar settings, and situations that require independent decision-making. As a result, Barran does not feel safe managing transport, navigating public spaces, or entering unfamiliar environments without support. Reduced safety awareness increases Barran's vulnerability to harm, exploitation, and crisis escalation, particularly in the absence of supervision and consistent support. His safety is highly dependent on structured assistance, predictable environments, and the presence of trusted support people to help assess risk and respond to safety concerns appropriately.

Safety Risk	Barran presents with elevated safety risks due to the combined impact of impaired cognition, emotional dysregulation, limited informal support, and a living environment he does not perceive as safe. These factors significantly reduce his ability to protect his own wellbeing and respond effectively to everyday risks without assistance. Cognitive overload and fear-driven thinking increase the likelihood of withdrawal, missed appointments, poor decision-making, and escalation into mental health crises. When overwhelmed, Barran disengaged from services or daily routines, increasing the risk of deterioration and delayed intervention. His lack of reliable informal support further limits opportunities for early identification of risk or timely assistance.
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SOCIAL	
Communication	Barran primarily communicates verbally and in writing. When emotionally regulated and feeling safe, he is articulate and capable of expressing complex ideas clearly and thoughtfully. However, this capacity fluctuates significantly depending on his stress levels, perceived safety, and overall mental state. During periods of anxiety or emotional dysregulation, he reports difficulty organising and clearly explaining his thoughts, often feeling misunderstood or dismissed by others. Under stress, Barran's communication becomes lengthy, repetitive, or highly detailed, reflecting attempts to ensure that he is believed and accurately understood. He experiences difficulty processing fast-paced conversations, group discussions, or information delivered verbally without written follow-up. In these situations, important details are missed or misunderstood, increasing frustration and distress. Barran also misinterprets tone, intent, or meaning, particularly in interactions involving authority figures, service providers, or unfamiliar people. These misinterpretations can quickly escalate emotional distress and contribute to withdrawal or heightened anxiety. Communication deteriorates further when his concerns are framed solely as paranoia rather than being explored in a trauma-informed and validating manner.

	Barran benefits most from slow-paced, clear communication, with information provided in writing, repeated as needed, and reinforced through trusted support people. This approach supports understanding, reduces miscommunication, and helps him remain emotionally regulated during interactions.
Social Interaction	Barran's social interaction is significantly limited by fear, mistrust, and a pervasive sense of being unsafe in his current circumstances. He reports a strong preference for being alone, not due to a lack of interest in relationships, but as a protective response to perceived threat. Over time, social interaction has become associated with danger rather than connection, particularly following repeated experiences of feeling targeted, dismissed, or harmed while living in unstable and unsupported environments. Barran experiences significant anxiety in social situations, especially when interacting with unfamiliar people, authority figures, or in settings where he feels observed, judged, or powerless. Social engagement is highly draining due to chronic hypervigilance, the emotional effort required to monitor safety, and cognitive overload associated with schizophrenia, trauma, and acquired brain injury. Barran finds it difficult to initiate and maintain friendships, particularly in his current location where he does not feel emotionally or physically safe enough to trust others. He reports long-standing social isolation and a progressive loss of meaningful connections, which has directly contributed to deterioration in his mental health, increased rumination, and reduced motivation. Barran is capable of forming meaningful and reciprocal relationships. This is demonstrated by his long-term relationship with his fiancé, whom he identifies as a primary protective factor. Prolonged separation from this relationship has significantly worsened his social withdrawal, loneliness, and sense of isolation.
Community Engagement	Barran is not currently engaged in community groups, clubs, religious organisations, or other structured community activities. He actively avoids

	community participation due to significant anxiety, fear for his personal safety, and perceived threats in public environments. These fears make it difficult for him to leave his accommodation or engage with others outside of controlled and familiar settings. Limited access to transport, financial restriction, and the absence of consistent support further reduce Barran's ability to access the community independently. As a result, he reports spending the majority of his time isolated within his accommodation. This prolonged isolation contributes to increased distress, rumination, and a gradual decline in mental health and overall functioning. With appropriate support in place, Barran has expressed interest in re-engaging with meaningful activities, particularly those related to writing, art, advocacy, and intellectual pursuits. However, meaningful community engagement will require consistent support worker assistance, emotional regulation support, and access to a psychologically safe and predictable environment. Without these supports, independent community participation is not currently achievable.
Social Vulnerability	Barran presents with high social vulnerability due to the combined impact of impaired judgement, fluctuating insight, a significant trauma history, schizophrenia-related perceptual disturbances, and prolonged social isolation. These factors reduce his ability to protect his own interests and increase reliance on others during periods of distress or instability. Barran reports repeated experiences of being taken advantage of emotionally, financially, and systemically, particularly during times when he lacked adequate support or advocacy. Barran describes being subjected to financial control and restriction under guardianship arrangements, which has resulted in an inability to access his own funds, pay bills, maintain essential technology, or meet basic daily needs. This has left him dependent on systems and others, significantly disempowered, and unable to safeguard himself from further harm. As a consequence of restricted finances, Barran reports losing access to email, phone, and computer services, which severely limited his ability to

	communicate, advocate for himself, and maintain social or professional connections. Barran reports multiple situations in which his concerns about personal safety, threats, and exploitation were dismissed or reframed as paranoia, rather than explored or investigated. These experiences have increased mistrust of services and reduced his willingness to seek help. When distressed or isolated, Barran finds it difficult to determine whether others are being genuine or manipulative, increasing vulnerability to exploitation. Barran also lacks confidence in setting boundaries, particularly with authority figures, service providers, and unfamiliar individuals, due to fear of retaliation, loss of services, or further restrictions. His behaviour and communication style have often been misunderstood, with expressions of distress interpreted as non-compliance or delusional thinking rather than indicators of unmet needs and fear. Together, these factors place Barran at high risk of further exploitation, neglect, emotional harm, and crisis escalation without consistent advocacy, supervision, and protective support in place.
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ACTIVITIES OF DAILY LIVING/SELF CARE		Level of support
Bathing/ Showering	- Barran is physically able to shower independently, however his functional capacity is significantly impacted by acquired brain injury, schizophrenia, ADHD, and chronic psychological distress. - Executive dysfunction related to brain injury and ADHD results in difficulty initiating and following through with showering without external prompting or support. - Barran frequently delays or avoids showering due to cognitive fatigue, reduced motivation, and emotional overwhelm associated with his mental health conditions.	Modified Independence – prompting assistance

	• During periods of heightened anxiety, paranoia, or perceived threat related to schizophrenia and trauma, self-care tasks are deprioritised as Barran becomes focused on safety concerns. • Trauma history and schizophrenia contribute to withdrawal and shutdown, during which basic hygiene is not initiated despite physical ability. • Barran goes several days without showering when sleep-deprived, distressed, or overwhelmed by NDIS, financial, or Public Guardian-related stressors.	
Toileting/ Continence	• Barran is physically independent with toileting and continence and does not require physical assistance for these tasks. • Continence is not directly impaired by Barran's disabilities, including acquired brain injury, schizophrenia, or ADHD. • However, during periods of severe psychological distress, cognitive overload, or disorganisation related to his mental health conditions, Barran show reduced attention to hygiene standards following toileting	Independent
Grooming	• Barran is physically capable of completing grooming tasks, including shaving, hair care, and nail trimming. • Consistency with grooming is reduced due to executive dysfunction and cognitive fatigue associated with acquired brain injury and ADHD. • During depressive or highly anxious phases, grooming tasks are frequently neglected due to low motivation and emotional distress linked to his mental health conditions. • Schizophrenia-related withdrawal contributes to reduced engagement in appearance-related	Independent

	self-care, particularly during periods of increased symptoms.	
Dressing	• Barran is physically able to dress independently, however functional performance is inconsistent due to the impact of acquired brain injury, schizophrenia, ADHD, and trauma-related distress. • Executive dysfunction and impaired task initiation result in wearing the same clothes repeatedly without recognising the need to change. • During periods of emotional distress and cognitive overload, Barran has reduced awareness of cleanliness, weather appropriateness, and the condition of clothing. • Financial hardship and loss of personal possessions, associated with systemic and mental health impacts, limit access to clean and appropriate clothing.	Modified independence - prompting assistance
Food Intake	• Barran is physically able to eat independently and does not require physical assistance with feeding. • Food intake is inconsistent due to the combined impact of financial hardship, anxiety, and executive dysfunction associated with acquired brain injury, schizophrenia, and ADHD. • Barran reports skipping meals during periods of heightened stress, fear, or emotional overwhelm, when mental health symptoms escalate. • Cognitive fatigue and impaired planning skills limit his ability to organise, access, and maintain regular, nutritious meals. • A history of food insecurity, linked to financial restriction and mental health instability,	Independent

	increases the risk of inadequate nutritional intake.	
Medication Management	• Barran is prescribed psychotropic and other medications for the management of schizophrenia, ADHD, and related mental health conditions. • Barran uses medication aids (e.g. pill organisers) when available; however, adherence is inconsistent due to the functional impact of his disabilities. • Memory impairment associated with acquired brain injury results in forgotten doses without external prompting or reminders. • ADHD-related difficulties with routine establishment and follow-through further affect consistent medication use. • Periods of anxiety, paranoia, and heightened distress related to schizophrenia can interfere with trust in medications and regular adherence. • Limited access to technology and health services reduces effective medication oversight and follow-up.	Modified independence - prompting assistance
Sleep	• Barran's sleep is significantly impaired due to the combined effects of schizophrenia, acquired brain injury, trauma-related hypervigilance, and chronic anxiety. • He experiences difficulty falling asleep and maintaining sleep, driven by paranoia, intrusive thoughts, and a persistent sense of threat. • Sleep deprivation exacerbates cognitive impairment, emotional dysregulation, and psychotic symptoms, directly impacting his functional capacity.	Impaired

	• Irregular sleep patterns contribute to reduced ability to manage self-care, regulate emotions, and engage in daily activities, particularly during periods of mental health instability. • A lack of a safe and stable living environment further worsens sleep quality and limits recovery.	
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INSTRUMENTAL ACTIVITIES OF DAILY LIVING

Education	Barran is not currently enrolled in any education, training, TAFE, or university program. Although he has a high premorbid level of academic achievement, including tertiary-level study, his current functional capacity is significantly reduced. The combined effects of acquired brain injury, schizophrenia, ADHD, and chronic psychological distress have resulted in marked difficulties with attention, memory, executive functioning, emotional regulation, and tolerance to stress, all of which interfere with his ability to engage in formal education. Barran is unable to independently manage enrolment processes, paperwork, online systems, deadlines, or attendance requirements. Cognitive fatigue and anxiety significantly limit his capacity to participate in structured learning environments, while social anxiety, paranoia, and fear of perceived threats make group-based settings feel unsafe and overwhelming. These factors prevent sustained participation and increase the risk of withdrawal or deterioration.
Employment	Barran is not currently working, volunteering, or engaged in paid employment. While he has a previous work history that includes advocacy and professional roles, he has been unable to maintain employment due to a significant deterioration in mental health and cognitive functioning. His current presentation reflects reduced capacity to meet the demands of a work environment. Symptoms associated with schizophrenia, ongoing anxiety, impaired concentration, and emotional dysregulation significantly impact his work

	readiness. Barran experiences difficulty following instructions, maintaining routines, coping with workplace stress, and sustaining attention across tasks. Barran also reports paranoia, fear of others, and difficulty interpreting intent, which make workplace environments feel unsafe and destabilising. As a result, he lacks the capacity to independently manage job seeking, interviews, workplace expectations, or conflict resolution.
Meal Preparation	• Barran is unable to independently plan, prepare, and cook meals consistently due to the functional impact of acquired brain injury, schizophrenia, ADHD, and chronic psychological distress. • Executive dysfunction, memory impairment, and cognitive fatigue significantly limit his ability to sequence tasks, remember steps, and use kitchen appliances safely. • Anxiety, paranoia, and emotional distress associated with schizophrenia and trauma lead to avoidance of cooking, particularly when he feels unsafe or overwhelmed. • Barran requires support worker assistance to prepare meals or provide full supervision with step-by-step prompting throughout the cooking process. • When involved in meal preparation, he needs continuous prompting, reassurance, and occasional physical assistance to complete tasks safely.
Cleaning	• Barran is unable to independently maintain a clean and safe living environment due to the functional impact of acquired brain injury, schizophrenia, ADHD, and chronic psychological distress. • Executive functioning impairment results in difficulty initiating household tasks, planning cleaning activities, and completing them to an adequate standard. • Cognitive overload and emotional distress related to his mental health conditions often lead to shutdown when he is faced with mess or multiple cleaning tasks.

	• Support workers complete the majority of household cleaning tasks on Barran's behalf to ensure basic hygiene and safety are maintained. • When involved in cleaning, Barran requires full supervision, verbal prompting, reassurance, and physical assistance to complete tasks.
Laundry	• Barran is unable to independently manage laundry tasks due to the functional impact of acquired brain injury, schizophrenia, and ADHD. • Memory impairment and inattention affect his ability to sort clean and dirty clothing, remember when laundry needs to be done, and sequence washing, drying, and folding steps. • Clothes are forgotten in washing machines or mixed incorrectly, reflecting cognitive and attentional difficulties rather than lack of effort. • Support workers complete laundry tasks or provide full supervision and step-by-step prompting when Barran is involved.
Shopping	• Barran is unable to shop independently due to the functional impact of schizophrenia, acquired brain injury, ADHD, and trauma-related anxiety. • Anxiety, paranoia, and fear of public environments significantly limit his ability to enter shops or remain in busy settings without distress. • Executive dysfunction affects planning, list-making, budgeting, and decision-making during shopping tasks. • When unsupervised, Barran is at risk of impulsive spending and poor food choices. • Support workers accompany Barran to shops, manage transactions, and provide constant guidance and reassurance throughout the process.
Finances	• Barran does not have the capacity to independently manage finances due to cognitive impairment, memory deficits, impaired judgement, and emotional dysregulation associated with acquired brain injury, schizophrenia, and ADHD. • Barran receives income through the Disability Support Pension (DSP).

	• Barran's finances are currently managed by the Public Trustee. • Barran reports experiencing severe financial hardship and debt, which he attributes to poor financial management and restricted access to his own funds. • He receives a limited weekly allowance, which is insufficient to meet basic living needs such as food, transport, and essential items. • Cognitive and executive functioning impairments prevent Barran from budgeting, paying bills, tracking expenses, or managing financial priorities independently. • Barran is highly vulnerable to financial exploitation without safeguards in place due to impaired judgement and fluctuating insight
Transport	• Barran holds a driver's licence and previously drove independently; however, he currently has no access to a vehicle as it was removed following police involvement and is held in a government holding yard. • Anxiety, paranoia, and perceived safety threats associated with schizophrenia and trauma make public transport unsafe and highly distressing for Barran. • Cognitive impairment related to acquired brain injury affects route planning, navigation, attention, and problem-solving during travel. • Due to these disability-related limitations, Barran is unable to attend appointments or access the community independently. • Support workers currently provide all transport for medical appointments, shopping, and essential activities.
Appointments	• Barran is unable to independently manage appointments due to the functional impact of acquired brain injury, schizophrenia, ADHD, and chronic psychological distress. • Memory impairment results in forgotten appointment dates, times, and locations without external reminders. • Reduced motivation and impaired task initiation occur during periods of emotional distress or cognitive fatigue, affecting follow-through.

	• Anxiety and paranoia interfere with attendance and engagement at appointments, particularly in unfamiliar or high-stress settings. • Support workers are required to schedule appointments, provide reminders, accompany Barran, and assist with communication with services.
Leisure	• Barran's leisure activities are very limited due to the functional impact of schizophrenia, acquired brain injury, ADHD, and chronic anxiety. • He enjoys painting, writing, watching television, and spending time with support workers, which are activities that are completed in safe and familiar environments. • Anxiety, low motivation, financial restriction, and fear of public spaces significantly limit engagement in hobbies or recreational activities outside the home. • Barran does not independently initiate leisure activities beyond basic, familiar routines due to reduced volition and executive dysfunction. • Social and community-based leisure activities are not accessed without the presence of a support worker to provide reassurance and structure. • Support workers play a key role in facilitating meaningful activity, providing motivation, and reducing social isolation.

CORE SUPPORTS

Assistance with daily life	Support Worker: Barran requires 5 hours per day of support worker assistance due to the cumulative functional impact of acquired brain injury, schizophrenia, ADHD, trauma-related mental health conditions, and chronic psychological distress. While Barran is physically capable in many areas, his ability to initiate, plan, sequence, and complete daily living tasks is significantly impaired by cognitive dysfunction, emotional dysregulation, paranoia, anxiety, memory impairment, and reduced stress tolerance. 1. Personal Care and Self-Care Support:
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	○ Prompting and supervision for showering, grooming, dressing, and hygiene ○ Monitoring personal presentation and cleanliness ○ Emotional reassurance during periods of distress or withdrawal 2. Meal Preparation, Nutrition, and Food Intake: ○ Planning meals and ensuring adequate nutrition ○ Meal preparation or full assistance with cooking ○ Prompting and monitoring food intake ○ Cleaning up after meals 3. Medication Management and Health Routines: ○ Prompting and supervising medication administration ○ Checking adherence and medication aids ○ Supporting daily health routines (sleep hygiene, hydration reminders) 4. Household Tasks and Daily Living Maintenance: ○ Cleaning, laundry, and maintaining a safe living environment ○ Assistance with organising personal belongings ○ Monitoring hygiene, food safety, and household risks 5. Daily Planning, Appointments, and Functional Organisation: ○ Scheduling and preparing for appointments ○ Providing reminders and accompanying Barran when required ○ Supporting communication with services ○ Assisting with daily planning, budgeting-related tasks, and task sequencing Frequency: 5 hours per day
Assistance with social and community participation	Support Worker: Barran requires 5 hours per day of support worker assistance to engage safely and meaningfully in social and community activities due to the functional impact of schizophrenia, acquired brain injury, ADHD, trauma-related mental health conditions, and chronic anxiety. These conditions significantly impair his ability to leave the home independently, interact with others, regulate emotions in public settings, navigate

community environments, and participate in social or purposeful activities without distress or risk escalation.

1. Supported Community Access and Outings:

- Accompanied community outings (e.g. walking, cafes, parks, errands)
- Support with transport and navigation
- Monitoring safety and responding to distress in public settings

2. Social Interaction and Relationship Support:

- Supported social interaction with others in the community
- Emotional support during interaction
- Assistance interpreting social cues and managing anxiety

3. Meaningful Activity and Skill-Based Engagement:

- Support to engage in meaningful activities such as writing, art, advocacy, or intellectual interests
- Encouragement and structure to initiate and sustain activities
- Support to access libraries, community centres, or safe interest-based environments

4. Appointments, Advocacy, and Community-Based Services:

- Accompaniment to community-based appointments or services
- Support with communication, advocacy, and emotional regulation
- Assistance navigating community systems

5. Emotional Regulation and De-escalation in Community Settings:

- Grounding and reassurance during community participation
- Monitoring for signs of overwhelm
- Supporting safe transitions back to the home environment

Frequency: 5 Hours per day

Transport

Barran will benefit from Level 3 of transport assistance.

CAPACITY BUILDING SUPPORTS

Support Coordination	Support Coordination Level 2 and 3: Barran has a highly complex psychosocial and neurological disability profile, including acquired brain injury, schizophrenia, ADHD, trauma history, emotional dysregulation, impaired cognition, and significant environmental and systemic stressors. He experiences difficulty engaging with services, managing change, advocating for himself, and coordinating support without distress or withdrawal. Barran's support environment involves multiple systems, including the NDIS, Public Guardian, Public Trustee, housing providers, mental health services, allied health professionals, and support workers. Interactions with these systems are a significant trigger for anxiety, paranoia, and emotional dysregulation. Without structured coordination, Barran is at high risk of service breakdown, disengagement, tenancy failure, and crisis presentation. Frequency: Level 3 for 30 hours per year and Level 2 for 50 hours per year
Improved Living Arrangements	Assistance and Accommodation and Tenancy Obligations: Barran requires ongoing assistance with accommodation and tenancy obligations due to the functional impact of acquired brain injury, schizophrenia, ADHD, trauma-related mental health conditions, and chronic psychological distress. These conditions significantly impair his ability to understand, manage, and comply with the practical, administrative, and interpersonal requirements of maintaining stable housing.
Increased Social and Community Participation	Assistance in Coordinating or Managing Life Stages: Barran requires assistance in coordinating and managing life stages due to the combined impact of acquired brain injury, schizophrenia, ADHD, trauma-related mental health conditions, and chronic psychological distress. These conditions significantly impair his ability to plan for change, adapt to new circumstances, and manage complex transitions across different areas of life.
Improved Relationships	Positive Behaviour Support:

	Barran requires 60 hours per year of Positive Behaviour Support due to a complex and high-risk psychosocial disability profile that includes acquired brain injury, schizophrenia, ADHD, trauma history, emotional dysregulation, and significant environmental stressors. Barran does not display outward aggression; however, his behaviour of concern is characterised by withdrawal, shutdown, avoidance, self-neglect, crisis escalation, and a documented history of self-harm and suicide attempt. These behaviours are strongly linked to unmet needs, perceived threat, lack of safety, loss of autonomy, and inadequate support rather than intentional or oppositional behaviour. Frequency: 60 hours per year
Improved Daily Living	Occupational Therapy : Barran requires 40 hours per year of Occupational Therapy due to the significant functional impact of acquired brain injury, schizophrenia, ADHD, and trauma-related mental health conditions. While Barran has physical capacity in many areas, his ability to initiate, plan, organise, sequence, and sustain everyday activities is severely impaired. Cognitive fatigue, memory deficits, emotional dysregulation, impaired executive functioning, anxiety, and paranoia significantly limit independence across daily living, community participation, and safety. 1. Comprehensive Functional Assessment: ○ Assessment of daily living skills, self-care, and routines ○ Cognitive and executive functioning assessment ○ Assessment of emotional regulation and sensory triggers ○ Evaluation of safety awareness, risk, and environmental barriers ○ Functional capacity assessment for NDIS reporting 2. Development of Individualised Intervention Plan: ○ Development of a tailored OT intervention plan ○ Goal setting in collaboration with Barran and support network ○ Identification of priorities across daily living, community access, and safety ○ Alignment with NDIS goals and supports

3. Skill Development and Routine Building:

- Supporting development of daily routines for self-care, meals, sleep, and medication
- Task simplification and sequencing strategies
- Use of visual aids, written prompts, and checklists
- Strategies to improve task initiation and completion

4. Environmental Assessment and Modification:

- Assessment of living environment for safety and sensory impact
- Recommendations to improve predictability, emotional safety, and accessibility
- Strategies to reduce triggers related to paranoia and hypervigilance
- Equipment or environmental recommendations where appropriate

5. Support Worker Training and Collaboration:

- Training support workers in OT strategies and routines
- Supporting consistent implementation of strategies across staff
- Liaison with PBS practitioners, support coordinators, and services

6. Review, Monitoring, and Reporting:

- Monitoring progress and adjusting interventions
- Review of goals and functional outcomes
- Preparation of progress reports and NDIS documentation

Frequency: 40 hours per year

Psychology:

Barran requires psychological support due to a complex mental health profile that includes schizophrenia, trauma history, adjustment disorder, anxiety, emotional dysregulation, and the psychological impact of an acquired brain injury. These conditions significantly affect his emotional wellbeing, coping capacity, thought processes, behaviour, and ability to engage safely with daily life, services, and the community.

Reasonable and Necessary:

Barran's capacity-building supports meet the NDIS Reasonable Necessity criteria as follows:

Occupational Therapy

- A. Occupational Therapy supports Barran's goals of independence, safety, and improved daily functioning by addressing difficulties with self-care, routines, memory, executive functioning, and environmental safety.
- B. OT assists Barran to develop practical skills and strategies that enable participation in daily activities, community access, and future engagement in meaningful roles. Improved functional capacity supports long-term participation and reduces isolation.
- C. OT focuses on building sustainable skills and routines that reduce long-term dependence on higher levels of support. Environmental modifications and functional strategies reduce ongoing support costs and risk.
- D. OT is a recognised, evidence-based intervention for individuals with acquired brain injury, mental illness, and cognitive impairment. It supports functional improvement, safety, and quality of life using established best practice.
- E. Barran's needs exceed what informal support can reasonably provide. OT delivers specialist assessment and intervention that cannot be substituted by family, carers, or the community.

Psychology

- A. Psychology supports Barran's goals of emotional stability, recovery, safety, and improved wellbeing. It assists him to manage trauma, anxiety, paranoia, low mood, and distress that directly interfere with daily life and goal attainment.
- B. Psychological intervention reduces emotional and cognitive barriers that prevent Barran from engaging with others, attending appointments, and participating in community life. Improved mental health supports broader participation.

- C. Psychology reduces reliance on crisis services, hospital admissions, and reactive interventions. Preventative therapeutic support is more cost-effective than managing repeated mental health crises.
- D. Psychological therapy is evidence-based and best practice for individuals with schizophrenia, trauma histories, and emotional dysregulation. It supports long-term stability, coping, and recovery.
- E. Barran does not have access to informal mental health support capable of addressing complex trauma and severe mental illness. Psychology provides professional treatment that cannot reasonably be provided by family or community members.

CAPITAL SUPPORT NEEDS

Assistive Technology	Nil.
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I hope you find this report helpful in developing a NDIS plan for Barran Dodget that supports his goals, meets his needs and promotes his safety, health and wellbeing.

If you have any questions regarding this report and subsequent recommendations, please do not hesitate to contact me on 0449571641 or stefan@advancedot.com.au

Name:	Title:	Date:	Signature:
Stefan Barbas	Occupational Therapist	13/02/2026	<i>Stefan Barbas</i>